

GLOBAL HEALTH & WELLNESS

1234 Medical Plaza, Metropolis, NY 10001
Phone: (555) 123-4567 | www.globalhealthcenter.example.com

COMPREHENSIVE MEDICAL RECORD

Patient Name:	Johnathan Alexander Doe	DOB / Age:	1982-05-12 / 42 Yrs
Gender:	Male	Blood Type:	O Positive
Height / Weight:	178 cm / 75.5 kg	BMI:	23.8
Address:	742 Evergreen Terrace	City / Country:	Chennai / Tamil Nadu
Phone:	+91 98765 43210	Emergency Contact:	+91 98765 43211 (Wife)

1. Chief Complaint & History of Present Illness

Patient presents today for a comprehensive annual physical examination and follow-up on previously diagnosed Type 2 Diabetes Mellitus and Essential Hypertension. Patient reports feeling generally well but notes occasional mild fatigue in the late afternoons over the past 3 months. No chest pain, shortness of breath, nausea, or vomiting. Patient maintains a moderate exercise routine consisting of brisk walking 3 times a week. Diet consists of moderately healthy meals but admits to occasional high-sugar snacks.

2. Past Medical History

- Type 2 Diabetes Mellitus - Diagnosed May 2018.
- Essential Hypertension - Diagnosed November 2020.
- Hyperlipidemia - Diagnosed 2019.
- Appendectomy - March 2010.
- Mild Asthma - Diagnosed 1995 (resolved).
- Seasonal Allergies (Pollen) - Active.

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3. Review of Systems

General: No recent weight loss, fever, or night sweats. Complains of mild afternoon fatigue.

Cardiovascular: No palpitations, orthopnea, or edema. Hypertension well controlled.

Respiratory: No chronic cough, wheezing, or shortness of breath.

Gastrointestinal: Appetite is good. No nausea, vomiting, diarrhea, or constipation. Regular bowel movements.

Musculoskeletal: No joint pain, swelling, or muscle weakness. Full range of motion in all extremities.

Neurological: No headaches, dizziness, numbness, or tingling. Normal gait and coordination.

Psychiatric: No depression, anxiety, or sleep disturbances.

Dermatological: No rashes, suspicious lesions, or changes in pigmentation.

4. Physical Examination

- Vitals: BP 122/80 mmHg | HR 72 bpm | Temp 98.6°F | RR 16 breaths/min | SpO2 99% on room air.
- General Appearance: Well-developed, well-nourished male in no acute distress.
- HEENT: Normocephalic, atraumatic. PERRLA, EOMI. Sclerae anicteric. Tympanic membranes clear bilaterally. Oropharynx clear without erythema or exudate.
- Neck: Supple. No thyromegaly, jugular venous distention, or carotid bruits. No lymphadenopathy.
- Cardiovascular: Regular rate and rhythm. Normal S1 and S2. No murmurs, rubs, or gallops. Pulses 2+ bilaterally in upper and lower extremities.
- Lungs: Clear to auscultation bilaterally. No wheezes, rales, or rhonchi. Symmetrical chest expansion.
- Abdomen: Soft, non-tender, non-distended. Bowel sounds active in all four quadrants. No hepatosplenomegaly or masses appreciated.
- Extremities: No clubbing, cyanosis, or edema. Calf supple, non-tender.
- Skin: Warm and dry. No rashes or suspicious moles. Normal capillary refill.
- Neurologic: Alert and oriented x 3. Cranial nerves II-XII intact. Sensation intact to light touch and pinprick globally. Deep tendon reflexes 2+ and symmetric.

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5. Assessment

- Type 2 Diabetes Mellitus: Currently well-controlled on Metformin. HbA1c is 6.8%, which is near the target range of $< 7.0\%$. Patient advised to continue dietary modifications and increase physical activity to mitigate afternoon fatigue.
- Essential Hypertension: Controlled on Lisinopril 10mg. Blood pressure today is 122/80, which is excellent. Will continue current management.
- Hyperlipidemia: Stable. Patient reports adherence to statin therapy. Lipid panel shows LDL within target range. Will recheck in 6 months.

6. Plan

- Continue Metformin 500mg BID for diabetes management.
- Continue Lisinopril 10mg daily for hypertension.
- Dietary Counseling: Emphasized a low-glycemic, Mediterranean-style diet to stabilize energy levels in the afternoon.
- Exercise Prescription: Recommended 150 minutes of moderate-intensity aerobic activity per week, plus strength training twice a week.
- Laboratory Follow-up: Comprehensive Metabolic Panel (CMP), Complete Blood Count (CBC), and Lipid Panel to be drawn in 6 months.
- Return to Clinic (RTC): Schedule a follow-up appointment in 6 months for a routine check, or sooner if symptoms worsen.

Electronically signed by Dr. Sarah Wilson, MD

Date: September 30, 2026